



GLIZID TABLET

Description: White, round, flat bevel-edged, top double-scored tablet.

Content:

Each tablet contains:

Gliclazide 80.0 mg

Indication:

For non-insulin dependant diabetes mellitus, where dietary management alone is insufficient.

Pharmacology:

Gliclazide is a sulphonylurea hypoglycaemic agent. It stimulates the secretion of insulin by functioning islet beta cells of the pancreas. However, when the insulin secretion falls again, the hypoglycaemic effect persists and may be due to inhibition of hepatic glucose production and increased sensitivity to any available insulin. Gliclazide is only effective for patients whose pancreas is capable of producing insulin. In addition to this pancreatic action, it has been demonstrated that gliclazide administration may improve the metabolic utilisation of glucose at a peripheral level.

Gliclazide is readily absorbed from the gastrointestinal tract and peak plasma concentration occur 2 - 8 hours after administration. Metabolism is extensive in the liver and all metabolites are devoid of hypoglycaemic activity. It is extensively bound to plasma proteins (85% - 97%). 60% - 70% of the dose is excreted in the urine and 10% - 20% in the faeces as metabolites. The elimination half-life of gliclazide is 10 - 12 hours.

Adverse effects:

Gastrointestinal - nausea, vomiting, headache, anorexia, diarrhoea, metallic taste usually mild and dose-dependent. **Skin** - rashes and pruritis may occur. Photosensitivity has been reported. **Hypoglycaemic reactions** - confusion, coma, seizures, cerebral oedema, tachycardia, sweating, tremor and ischaemic chest pains may occur. **Hypersensitivity reactions** - manifestations of it may include cholestatic jaundice, leucopenia, thrombocytopenia, aplastic anaemia, agranulocytosis, hemolytic anaemia, erythema multiforme, Stevens-Johnson syndrome, exfoliative dermatitis and erythema nodosum. **Syndrome of inappropriate secretion of antidiuretic hormone (SIADH)** characterised by water retention, hyponatremia and central nervous system (CNS) effects may infrequently occur.

Precautions:

Dosage adjustments may be required in patients suffering intercurrent infections, trauma, shock or after anaesthesia. Reduction of dosage may be necessary in elderly patients and patients with renal dysfunction because of decreased metabolism and excretion. Care is necessary during excessive exercise as hypoglycaemia may be provoked.

Pregnancy & Lactation:

Usage in pregnancy : kindly refer to contraindication. Usage in lactation: risk benefits should be considered when used on lactating mothers as it is not known whether gliclazide is excreted in the breast milk as of other sulphonylureas.

Drug Interactions:

Compounds that may diminish the hypoglycaemia effect and requires an increase in dosage adjustment - rifampicin, thiazide diuretics, adrenaline, chlorpromazine, corticosteroids, cyclophosphamide, diazoxide, gemfibrozil, isoniazid, oestrogens oral contraceptives, phenytoin and thyroid hormones. Compounds that may increase the hypoglycaemic effect and requires a reduction in their dosage adjustment - anti-infective agents such as chloramphenicol, fluconazole, miconazole and sulphonamides, azapropazone, indobufen, phenylbutazone and salicylates, coumarin anticoagulants and heparin, lipid regulating agents such as clofibrate and halofenate; and monoamine oxidase inhibitors, captopril, cimetidine, doxepin, enalapril, fenfluramine, methyl dopa, nortriptyline, ranitidine and sulphapyrazine, anabolic steroids, isoniazid, mebendazole and tetracycline.

Beta blockers may mask symptoms of hypoglycaemia and may inhibit normal physiological response to hypoglycaemia.

Contraindications:

It is contraindicated in the presence of insulin dependant diabetes mellitus (e.g. juvenile diabetes), ketoacidosis, severe infections, stress, trauma, severe renal & hepatic impairment, adrenal or thyroid dysfunction, patients with sulphonylurea intolerance, pregnancy, unstable diabetes, diabetic pre-coma and coma.

Dosage and administration:

The dosage is determined by the physician according to the severity of the diabetic state. 40 - 80mg daily with meals, gradually increased, if necessary, up to 320mg daily. Doses of more than 160mg daily should be given in 2 divided doses.

Use in elderly : Reduction of dose may be necessary because of decreased metabolism and excretion. Dosage may be started at 40 mg twice daily and adjusted to usual adult dose with constant monitoring.

Use in children : Safety and efficacy has not been established.

Treatment of over dosage:

Symptoms : Hypoglycaemic reactions e.g. confusion, coma, seizures, cerebral oedema, tachycardia, sweating, tremor and ischaemic chest pains may occur.

Treatment : Gastric lavage and correction of the hypoglycaemia by the administration of intravenous glucose (50%). Blood sugar should be continuously monitored until effect of the drug has ceased. A prolonged infusion may be necessary. Possibility of renal dysfunction should be noted.

Storage Conditions : Keep container tightly closed. Store in a cool, dry place below 30°C. Protect from light. Keep away from children.

Presentation : 50 x 10's/blister

Shelflife : 3 years.

Manufactured & : **PRIME PHARMACEUTICAL SDN. BHD.**

Distributed by : 1505, Lorong Perusahaan Utama 1, Taman Perindustrian Bukit Tengah, 14000 Bukit Mertajam, Penang, Malaysia.